

BUMRUNGRAD INTERNATIONAL HOSPITAL

Heart Center — Inpatient Medical Report

33 Sukhumvit Soi 3 (Soi Nana Nua), Wattana, Bangkok 10110, Thailand

Tel: +66 (0) 2066 8888 | Fax: +66 (0) 2066 8899 | www.bumrungrad.com

CONFIDENTIAL MEDICAL REPORT

Patient Name:	Margaret Elaine CHEN	Date of Birth:	14 May 1967
Nationality:	British	Passport:	GB-982451637
Hospital Ref:	BIH-IPD-2026-030847	Room:	1204, 12th Floor
Admission Date:	27 February 2026, 14:32	Attending:	Dr. Siriporn Amara
Report Date:	28 February 2026	Status:	Inpatient — Cardiac Monitoring

PRESENTING COMPLAINT

58-year-old British female presented to Emergency Department on 27 February 2026 at 14:32 local time following sudden onset chest pain and near-syncope at the Jim Thompson House Museum, 6 Soi Kasemsan 2, Rama 1 Road, Bangkok. Patient was transported by ambulance from the Siam Kempinski Hotel (991/9 Rama 1 Road). Transit time: 22 minutes.

MEDICAL HISTORY

Essential hypertension diagnosed 2014, managed with **Amlodipine 10mg OD** and **Losartan 50mg OD**. Last cardiology review: Royal Brompton Hospital, London, 12 January 2026 — notes indicate well-controlled BP (128/82 mmHg), no evidence of end-organ damage, cleared for travel. No prior cardiac events. No diabetes. Non-smoker. Occasional alcohol. BMI 26.3.

INVESTIGATIONS ON ADMISSION

Investigation	Result	Reference Range	Flag
Troponin-I (high sensitivity)	0.89 ng/mL	< 0.04 ng/mL	CRITICAL HIGH
BNP (B-type natriuretic peptide)	487 pg/mL	< 100 pg/mL	HIGH
Creatinine	1.1 mg/dL	0.6 - 1.2 mg/dL	Normal
eGFR	62 mL/min/1.73m ²	> 60 mL/min/1.73m ²	Borderline
Total Cholesterol	228 mg/dL	< 200 mg/dL	HIGH
LDL Cholesterol	148 mg/dL	< 100 mg/dL	HIGH
HbA1c	5.4%	< 5.7%	Normal
INR	1.0	0.8 - 1.2	Normal
D-Dimer	0.42 mg/L FEU	< 0.50 mg/L FEU	Normal

ECG: ST-segment depression in leads V4-V6. No ST elevation. Normal sinus rhythm at 78 bpm.

Echocardiogram (28 Feb): Mild left ventricular systolic dysfunction. Ejection Fraction 45% (previously 62% per Royal Brompton records, Jan 2026). Mild mitral regurgitation. No pericardial effusion.

Chest X-ray: Mild pulmonary congestion. No consolidation. Heart size upper limit of normal.

DIAGNOSIS

Primary: Non-ST-elevation myocardial infarction (NSTEMI), likely secondary to hypertensive crisis.

Secondary: Essential hypertension (chronic, previously controlled). Hyperlipidaemia (newly identified).

CURRENT MANAGEMENT

IV Heparin infusion (titrated to aPTT). Dual antiplatelet therapy: Aspirin 100mg OD + Ticagrelor 90mg BID. Atorvastatin 40mg OD (newly initiated). Amlodipine 10mg OD (continued). Losartan 50mg OD (continued). Bisoprolol 2.5mg OD (newly initiated). Continuous cardiac monitoring. Bed rest with supervised mobilisation.

PLANNED PROCEDURE

Coronary angiography scheduled for **3 March 2026** at 09:00 via right radial access. Potential ad-hoc PCI (percutaneous coronary intervention / stenting) if indicated.

FITNESS TO FLY ASSESSMENT

OPINION: UNFIT FOR AIR TRAVEL

Patient is haemodynamically stable but remains in the acute phase of NSTEMI. Cabin pressure changes at altitude (equivalent to 6,000-8,000 ft) pose significant risk of cardiac decompensation, arrhythmia, and potential re-infarction. I strongly advise against any form of air travel for a **minimum of 10-14 days post-angiography/intervention**, pending satisfactory recovery and repeat echocardiographic assessment. Patient is stable but fragile.

Dr. Siriporn Amara, MD, FACC

Interventional Cardiologist, Bumrungrad Heart Center

Thai Board of Cardiology | Fellowship, Johns Hopkins Hospital, Baltimore

Thai Medical Council Licence: ■.29847